

PATIENT DEMOGRAPHICS & VISIT INFORMATION

Field	Detail
Date of Visit	March 20, 2026
Patient Age	71 years
Sex	Male
Weight	82 kg
Renal Function	eGFR 58 mL/min/1.73 ² (CKD Stage 3a)
Encounter Type	Office Visit — Primary Care
Attending Physician	[Physician Name]
Facility	[Facility Name]

SECTION 1: CLINICAL ENCOUNTER NOTE (SOAP FORMAT)

S — Subjective

71-year-old male presenting with right knee pain secondary to known osteoarthritis, rated 7/10 in severity and significantly affecting ambulation. Patient reports progressive functional limitation with activities of daily living. He has previously completed a six-week trial of acetaminophen 1,000 mg three times daily, which provided inadequate analgesic relief. He declines referral for physical therapy and is not a candidate for COX-2 inhibitor therapy due to cost constraints. Topical anti-inflammatory agents have been trialed previously without benefit. Patient was engaged in a thorough discussion of all analgesic options available to him.

Active problem list (relevant to today's visit):

- Right knee osteoarthritis — primary presenting complaint
- Non-valvular atrial fibrillation — on anticoagulation with warfarin (INR 2.4, therapeutic, checked 8 days ago)
- Hypertension — managed with lisinopril 10 mg daily
- Hypercholesterolemia — managed with atorvastatin 40 mg daily
- CKD Stage 3a — eGFR 58 mL/min/1.73²

O — Objective

Active Medication	Dose / Route / Frequency	Indication
Warfarin sodium	5 mg orally once daily	Stroke prevention, AF
Metoprolol succinate	50 mg orally once daily	Rate control, AF
Lisinopril	10 mg orally once daily	Hypertension

CLINICAL ENCOUNTER DOCUMENTATION

Primary Care Visit

CONFIDENTIAL

Date: March 20, 2026

Atorvastatin

40 mg orally once daily

Hypercholesterolemia

Most recent INR: 2.4 (therapeutic range; obtained 8 days prior to this visit)

Allergies: [To be completed per chart]

Vitals: [To be completed at time of visit]

A — Assessment

1. Right knee osteoarthritis with inadequate pain control on acetaminophen monotherapy.
2. Non-valvular atrial fibrillation, stable, on therapeutic anticoagulation.
3. CKD Stage 3a — renal function requires ongoing monitoring in context of new analgesic regimen.
4. Hypertension — blood pressure management may be affected by NSAID introduction; monitoring warranted.

P — Plan

Following a detailed risk-benefit discussion (documented in Section 2 below), the decision was made to initiate naproxen sodium at the lowest effective dose. Patient was counseled on all significant risks and verbally consented to proceed (see Section 4). The following plan was established:

- Initiate naproxen sodium 220 mg orally twice daily with food.
- Use at the lowest effective dose and for the shortest duration necessary to achieve adequate analgesia.
- Initiate proton pump inhibitor (PPI) therapy concurrently for gastrointestinal protection given anticoagulant co-administration: [specify PPI and dose].
- INR check within 5–7 days of naproxen initiation; thereafter per enhanced monitoring schedule (Section 3).
- Renal function panel (creatinine, BUN, eGFR) within 1–2 weeks of initiation.
- Blood pressure check within 1–2 weeks of initiation.
- Patient instructed to discontinue naproxen and seek immediate evaluation if signs of GI bleeding, abnormal bruising, or significant change in urine output occur (see patient counseling, Section 4).
- Follow-up appointment in 2–4 weeks, or sooner if concerns arise.

SECTION 2: RISK-BENEFIT ANALYSIS STATEMENT

Clinical Rationale

This patient presents with right knee osteoarthritis causing significant pain (7/10) and functional impairment. Conservative first-line pharmacotherapy with acetaminophen 1,000 mg TID over six weeks produced inadequate relief. Non-pharmacological alternatives including physical therapy have been declined. COX-2 inhibitor therapy was considered but is not feasible due to cost. Topical diclofenac has been trialed previously without benefit. Intra-articular injection was discussed as an option.

Oral NSAID therapy with naproxen sodium represents the next appropriate step in the analgesic ladder for this patient's condition, balanced against the documented risks below.

Identified Risks and Mitigating Factors

HIGH PRIORITY — Warfarin + NSAID Interaction

Naproxen potentiates anticoagulant effect of warfarin through competitive protein displacement and additive inhibition of platelet function, increasing risk of serious bleeding events. Patient's current INR is therapeutic at 2.4. Enhanced INR monitoring has been ordered (see Section 3). Patient instructed on signs of bleeding requiring immediate evaluation. Concurrent PPI ordered for GI mucosal protection.

MODERATE-HIGH PRIORITY — Renal Function (CKD Stage 3a, eGFR 58)

NSAIDs inhibit prostaglandin-mediated afferent arteriolar vasodilation, a mechanism increasingly relied upon as renal reserve declines. Risk of acute-on-chronic kidney injury is elevated. Patient is concurrently on lisinopril (ACE inhibitor), which independently reduces efferent arteriolar tone — this dual hemodynamic effect requires close monitoring. Renal function panel has been ordered within 1–2 weeks of initiation.

MODERATE PRIORITY — ACE Inhibitor Antihypertensive Attenuation

NSAIDs promote sodium and water retention, which can blunt the antihypertensive effect of lisinopril. Blood pressure monitoring within 1–2 weeks of naproxen initiation has been ordered. Lisinopril dose adjustment may be required.

Alternatives Considered and Declined

Alternative	Reason Not Selected
Topical diclofenac	Previously trialed — no benefit reported by patient
COX-2 inhibitor (e.g., celecoxib)	Not feasible — cost barrier identified by patient
Physical therapy	Declined by patient
Intra-articular corticosteroid injection	Discussed; patient and physician elected pharmacological approach first
Duloxetine	Discussed as an option; patient and physician elected naproxen as next step
Continued acetaminophen monotherapy	Inadequate relief demonstrated over six-week trial

Conclusion

Having exhausted or excluded lower-risk analgesic options, and following a thorough discussion of all identified risks and proposed monitoring strategies with the patient, the prescribing physician concluded that the benefit of improved pain control and functional mobility outweighs the identified risks at the lowest effective dose with enhanced monitoring. This decision reflects the patient's informed preferences and values as expressed during today's encounter.

SECTION 3: MONITORING PLAN**INR Monitoring Schedule**

Naproxen can affect INR unpredictably. Enhanced monitoring is required following initiation:

Timepoint	Test Required	Action Threshold
5–7 days post-initiation	INR	If INR >3.0: hold naproxen, reassess; adjust warfarin if required
2–3 weeks post-initiation	INR	Adjust warfarin dose as needed to maintain INR 2.0–3.0
4–6 weeks post-initiation	INR	Confirm stability; resume standard monitoring interval if stable
Ongoing (while on naproxen)	INR every 4 weeks	Any INR outside 1.8–3.2 — clinical review and dose adjustment
If naproxen is discontinued	INR within 5–7 days	Warfarin dose may require upward adjustment

Renal Function Monitoring

Timepoint	Test Required	Action Threshold
1–2 weeks post-initiation	Creatinine, BUN, eGFR	eGFR decrease >25%: discontinue NSAID, urgent review
4–6 weeks post-initiation	Creatinine, eGFR	Continued decline: nephrology referral
Ongoing every 3 months	eGFR	Trend monitoring for CKD progression

Blood Pressure Monitoring

Timepoint	Action
1–2 weeks post-initiation	BP check in office or patient-reported home reading
Ongoing	Standard hypertension monitoring; adjust lisinopril if persistent elevation

GI Bleeding — Signs and Symptoms to Monitor

Patient has been counseled to discontinue naproxen and seek urgent/emergency evaluation for the following:

- Black, tarry, or bloody stools (melena or hematochezia)
- Vomiting blood or material resembling coffee grounds
- Unusual or unexpected bruising — particularly larger ecchymoses
- Prolonged bleeding from minor cuts or wounds
- Abdominal pain, cramping, or epigastric discomfort that is new or worsening
- Significant fatigue, pallor, or dizziness that may indicate occult blood loss
- Dark or cola-coloured urine (may indicate haematuria)
- Significant decrease in urine output — potential sign of NSAID-associated nephrotoxicity

Concurrent PPI Therapy

A proton pump inhibitor has been prescribed concurrently with naproxen for gastrointestinal mucosal protection. Medication details to be completed by prescriber: [PPI name, dose, frequency].

SECTION 4: PATIENT COUNSELING DOCUMENTATION & CONSENT

Counseling Provided

The following topics were discussed with the patient in plain language during today's visit. The patient demonstrated understanding and had the opportunity to ask questions, which were answered to his satisfaction.

Topics covered:

- Nature of right knee osteoarthritis and rationale for escalating pain management
- Mechanism and purpose of naproxen sodium; expected onset and duration of effect
- Increased risk of bleeding when combined with warfarin, including potential GI, intracranial, and systemic hemorrhage
- Importance of INR monitoring and adherence to the enhanced monitoring schedule
- Signs and symptoms of GI bleeding requiring urgent evaluation (enumerated in Section 3)
- Renal risk associated with NSAID use in the context of CKD and concurrent ACE inhibitor therapy
- Potential for blood pressure elevation and attenuation of lisinopril efficacy
- Instruction to take naproxen with food and/or a full glass of water
- Importance of not exceeding the prescribed dose or combining with other NSAIDs or aspirin without physician guidance
- Concurrent PPI prescription explained — purpose and instructions provided
- All available alternatives were discussed and documented in Section 2; patient's reasons for declining alternatives were noted
- Patient instructed to contact the office or seek emergency care for any concerning symptoms

Patient's Expressed Understanding and Decision

The patient verbally confirmed understanding of the risks associated with naproxen sodium in the context of his medication profile and renal function. He acknowledged the alternatives discussed, confirmed prior trial and failure of lower-risk options, and expressed his preference to proceed with naproxen at the lowest effective dose. The patient demonstrated decision-making capacity and expressed his consent to the proposed treatment plan freely and without coercion.

Consent Documentation

Consent Item	Status
Risks of naproxen + warfarin interaction explained	Confirmed — patient verbally acknowledged
Bleeding signs and action plan reviewed	Confirmed — patient verbally acknowledged
Renal monitoring plan explained	Confirmed — patient verbally acknowledged
BP monitoring plan explained	Confirmed — patient verbally acknowledged
Alternative therapies discussed and declined	Confirmed — documented in Section 2

CLINICAL ENCOUNTER DOCUMENTATION

Primary Care Visit

CONFIDENTIAL

Date: March 20, 2026

Patient expressed understanding	Confirmed
Patient consented to proceed	Confirmed — verbal consent obtained
Decision-making capacity assessed	Intact

Physician Attestation

I attest that the above clinical encounter note, risk-benefit analysis, monitoring plan, and patient counseling documentation accurately reflect the care provided and discussions held during today's visit. The clinical decision to prescribe naproxen sodium was made following a comprehensive review of the patient's medication profile, comorbidities, and informed refusal of lower-risk alternatives, consistent with my professional judgment and the patient's expressed preferences.

Physician Signature
[Physician Name, Credentials]

Date & Time
March 20, 2026

This document constitutes a legal medical record. Any amendments must be dated, timed, and signed by the attending physician. Unauthorized alteration is prohibited.